INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pembrolizumab 400 mg · every 6 weeks · lung cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
42 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00558
Version 13

One treatment visit every 6 weeks; your team confirms duration.

Treatment stages

01 **Pembrolizumab every 6 weeks**
One immunotherapy medicine

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Pembrolizumab 400 mg · every 6 weeks · lung cancer
National Cancer Control Programme (NCCP) 00558

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/ blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

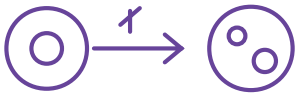
How pembrolizumab works

For metastatic lung cancer, the aim is to control cancer and help people live longer. After surgery and platinum chemotherapy, the adjuvant aim is to reduce the chance of cancer returning. Ask which aim applies to you.

IMMUNOTHERAPY

Pembrolizumab

Keytruda



Blocks an immune brake

Pembrolizumab is an antibody that blocks PD-1, a control switch on immune T cells. Cancer cells can use this switch to reduce immune attack. Blocking it can help the immune system recognise and attack cancer. The same increased immune activity can inflame healthy organs, which is why new symptoms need attention even after treatment ends.

Immune cell → PD-1 blocked → Cancer attacked

What to watch for

Report new breathing, bowel, skin, hormone, heart or nerve symptoms promptly; these may be immune inflammation.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your lung cancer and treatment aim

First confirm which lung setting applies to you.

Metastatic treatment

Metastatic means the cancer has spread. First-line means the first medicine treatment for this stage. Here, pembrolizumab alone is offered for suitable cancers with a high PD-L1 result and without EGFR or ALK alterations. The aim is usually control, symptom relief and more time.

After surgery

Adjuvant means treatment after surgery to reduce the chance of cancer returning. This pathway follows complete resection and platinum chemotherapy for cancer at high risk of recurrence. It does not use the same high PD-L1 requirement as the metastatic pathway.

Your alternatives

Ask which other treatments, clinical trials, surveillance or symptom-focused care fit your exact cancer and health. Supportive care can help during either pathway.

How pembrolizumab works

This is an immunotherapy medicine.

Why healthy organs can be affected

The immune system can also attack healthy tissues. Lung, bowel, liver, kidney, skin, gland, heart and nerve inflammation can range from mild to serious. Some hormone changes need long-term replacement.

A pause is a safety decision

Your team may delay a dose, treat side effects or stop pembrolizumab. The usual response to immune toxicity is to hold or stop treatment, not to give a smaller dose. Feeling better or having normal tests does not by itself mean that pembrolizumab can restart. Your team reviews what happened, recovery and the steroid or hormone plan. Some reactions mean that treatment must remain stopped, even if the symptoms have settled. Follow the prescribed plan and never restart treatment or change steroids yourself. Restart depends on the original reaction, recovery and any steroid or hormone plan. Some reactions mean treatment must remain stopped even after symptoms settle.

Why this treatment may be offered

In a trial of suitable people with metastatic lung cancer and a high PD-L1 result, pembrolizumab helped people live longer than chemotherapy. After surgery and chemotherapy, a separate trial found a lower risk of recurrence or death. Its final long-term analysis did not demonstrate a statistically significant improvement in overall survival. The aims and evidence differ between these settings.

These results describe groups of people, not what will happen to one person. The metastatic and after-surgery studies answer different questions.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment and reviews

Pembrolizumab 400 mg · every 6 weeks · lung cancer

42-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ONE IMMUNOTHERAPY MEDICINE

Pembrolizumab every 6 weeks

C1 C2 C3 …

After complete surgery and platinum chemotherapy, the adjuvant course lasts up to 12 months. For metastatic cancer, treatment continues while it remains helpful and tolerable; the team may consider stopping after 24 months of sustained response without progression, or continuing when appropriate.

Your team confirms your treatment aim, duration and review plan. Treatment may change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

Checks across your treatment

This is a relative treatment map, not a booking calendar. Ask who books each test and reviews the result. Extra tests may be needed for symptoms.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00558 v13 · official regimen

THE WHOLE TREATMENT JOURNEY

Pembrolizumab 400 mg · every 6 weeks · lung cancer: treatment and checks

Lung cancer only: first-line metastatic non-small-cell lung cancer (NSCLC) or adjuvant NSCLC after surgery and platinum chemotherapy

Pembrolizumab 400 mg is given as a drip over about 30 minutes on Day 1 every 6 weeks (42 days). The hospital visit is longer because of checks and preparation. After complete surgery and platinum chemotherapy, the adjuvant course lasts up to 12 months. For metastatic cancer, treatment continues while it remains helpful and tolerable; the team may consider stopping after 24 months of sustained response without progression, or continuing when appropriate.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First infusion	Between infusions	Course review	After treatment
Treatment NCCP 00558	Confirm metastatic or adjuvant indication	400 mg intravenous (IV) over about 30 minutes	Day1 every 6 weeks while prescribed	Adjuvant maximum 12 months; metastatic individual review	Follow-up/ surveillance or next treatment
Blood count, kidney, liver and glucose NCCP 00558	Baseline blood tests	Results reviewed before dosing	Before every 6-week cycle	Confirm cause of abnormal tests; clinician decides hold, restart or stop	Follow unresolved abnormalities
Thyroid and other hormones NCCP 00558 · SMPC	Baseline thyroid function; other hormones if needed	Review symptoms and hormone medicines	Thyroid every 3–6 weeks; cortisol if clinically indicated	Confirm replacement and steroid-taper plan	Continue hormone follow-up if needed
Symptoms and immune effects NCCP 00558 · SMPC	Discuss existing illnesses and warning symptoms	Tell the nurse immediately about a reaction	Report new symptoms on any day	Hold/restart decisions need clinical review	Late immune effects remain possible
Cancer response and course length NCCP 00558	Confirm metastatic or adjuvant plan	Agree how benefit will be assessed	Scans and review on your team's schedule	Adjuvant: up to 12 months; metastatic: individual 24-month review	Agree surveillance or next treatment

What each check is for

1 Before first dose

Before starting

Confirm the indication and baseline safety

Ask / act: Confirm results, access, consent discussion, alert card and responsible teams.

2 Course-length decision

At planned course review

Metastatic and adjuvant durations differ

Ask / act: Document whether treatment ends, pauses or continues, and how follow-up is arranged.

3 **After the last dose**

After treatment

Immune effects and hormone needs can continue

Ask / act: Provide the monitoring and urgent-contact plan.

4 **Abnormal liver or kidney results**

Whenever a result changes

A blood result alone does not establish immune inflammation

Ask / act: Contact the treating team and confirm the investigation and treatment plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments

STEP 2

Course length

STEP 3

Checks between visits

Blood tests are due before each infusion. Thyroid tests stay every 3–6 weeks regardless of dose schedule. Ask about travel, work, transport and whom to contact if a test or visit is delayed.

STEP 4

Home medicines and records

Keep the alert card, symptom record, current medicine list and urgent contact details accessible. Your team may prescribe steroids or hormone replacement only if needed.

STEP 5

After the final dose

Keep reporting new symptoms. Some hormone medicines and monitoring continue after the cancer treatment finishes.

Your treatment course

The appointment pattern and total duration are different decisions.

Infusion schedule

Changing schedules

The lung trials below used 200 mg every 3 weeks. The 400 mg every 6 weeks schedule is supported by drug-exposure modelling and regulatory bridging; it was not the randomized schedule in those lung trials.

Tests and reviews

Tell your team about symptoms even when recent blood tests were normal.

Before starting

Your team checks the cancer diagnosis, treatment aim and relevant tumour tests. Blood count, kidney/liver, glucose and thyroid tests provide a baseline. Discuss existing illnesses, medicines, pregnancy and breastfeeding.

During treatment

Blood count, kidney/liver function and glucose before every 6-week cycle; thyroid every 3–6 weeks, cortisol if indicated, and symptom review throughout and after treatment.

During a pause or after stopping

Extra tests may be needed for symptoms. Follow the agreed steroid or hormone plan. Keep your alert card and continue reporting symptoms after the last dose.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Infusion	 **Day 1** Day 1 · hospital infusion
Immune effects	 **Days 1–42** Any day · also after treatment ends
Energy, skin and bowels	 **Days 1–42** Timing varies · report changes promptly

Symptoms can occur at any time: Treatment days and symptom timing are different: there is no reliably safe week for immune effects.

Confirm your treatment aim and duration: After complete surgery and platinum chemotherapy, the adjuvant course lasts up to 12 months. For metastatic cancer, treatment continues while it remains helpful and tolerable; the team may consider stopping after 24 months of sustained response without progression, or continuing when appropriate.

Why these bands stay broad: The next Day 1 is normally 42 days after this cycle's Day 1, if your team agrees it is safe to proceed. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed "safe days". Immune effects can happen on any day and after treatment ends. Carry your immunotherapy alert card and tell any healthcare team that you receive pembrolizumab.

Between-treatment notes

Day 1: Your infusion visit

The team reviews your blood results, symptoms and medicines. Tell the nurse straight away about itching, flushing, a tight chest, dizziness or feeling suddenly unwell during the drip. Before leaving, confirm the next tests, treatment visit and urgent contact number.

Day 2–42: Between visits

You may feel well between infusions, but immune inflammation can begin at any time. Keep a short symptom record and take medicines exactly as prescribed. Contact your team about new symptoms instead of waiting for your next appointment. Confirm thyroid tests are booked every 3–6 weeks, even if infusions are six weeks apart.

Support around the medicines

1 Sickness medicines

Pembrolizumab alone has a minimal risk of causing sickness. Routine anti-sickness medicines or infusion premedicines are not usually needed; your team can prescribe symptom relief if required.

2 Immune alert card

Carry your alert card during treatment and afterwards. Show it when seeking urgent care; symptoms can be mistaken for an infection or an ordinary stomach upset.

3 Hormones and steroids

Some immune effects need steroids or hormone replacement. Follow the prescribed plan and do not stop steroids suddenly. Ask for help if symptoms worsen or you cannot take essential medicines. If treatment is paused because of an immune side effect, your team checks recovery and any steroid or hormone plan before deciding whether it can restart. Do not change steroids or restart treatment yourself.

4 Pregnancy and breastfeeding

Discuss pregnancy, fertility and breastfeeding before treatment. Use effective contraception during treatment and for at least four months after the last dose. If pregnancy is possible or occurs, contact your oncology team promptly.

Practical support and everyday life

Keep your team informed about what matters to you.

Energy and activity

Fatigue can affect daily activities. Balance activity with rest and ask for help if tiredness is new, marked or accompanied by dizziness, headache, thirst or weakness; it may need medical assessment.

Eating, drinking and bowels

Drink as advised and report ongoing vomiting or diarrhoea. Do not assume diarrhoea is a minor stomach upset; your team needs to check for immune inflammation and other causes.

Medicines and pregnancy

Bring a full medicine list, including supplements and steroids. Ask before starting new medicines. Use the contraception plan and discuss fertility or breastfeeding with your team.

When to get urgent help

Use the contact plan given by your oncology service.

During the drip

Tell the nurse immediately if you become itchy, flushed, breathless, dizzy, tight-chested or suddenly unwell.

Care after treatment ends

Immune inflammation can start late. Tell any healthcare team about pembrolizumab even after the course has finished.

When heart, muscle and nerve symptoms overlap

Heart, muscle and nerve problems can occur together. New muscle pain or weakness with changed vision, unusual tiredness, breathlessness, chest pain or an irregular heartbeat needs immediate medical advice. Do not wait for several symptoms to appear together. Severe breathing difficulty, collapse or rapidly worsening weakness needs emergency care.

Questions worth asking

Is my treatment for metastatic cancer or to reduce recurrence after surgery?

Ask your team to write down the treatment aim. For metastatic disease, the aim is usually cancer control, symptom relief and more time. After complete surgery and platinum chemotherapy, adjuvant treatment aims to reduce the chance of recurrence.

Which tumour-test results support this plan?

For the first-line metastatic monotherapy pathway, the source requires high PD-L1 expression and no EGFR mutation or ALK translocation. The adjuvant pathway has different eligibility. Ask which results apply to your own cancer.

How long is my course, and what could change it?

The adjuvant course lasts up to 12 months. Metastatic treatment continues while helpful and tolerable; a 24-month review may lead to stopping or continuing. Symptoms, response and your preferences can change the plan.

Who arranges blood and thyroid tests between visits?

The oncology team arranges and reviews tests. Blood count, kidney/liver function and glucose are checked before each cycle; thyroid tests are usually every 3–6 weeks. Ask whether an extra test is needed between six-weekly infusions.

Which symptoms should I report straight away, and whom do I contact out of hours?

Report new or worsening breathing problems, persistent diarrhoea, severe abdominal pain, yellow eyes, reduced urine, severe headache, weakness, thirst or a spreading rash promptly. Keep the local oncology and out-of-hours numbers available; severe breathing difficulty, collapse, seizure or severe chest pain needs 112 / 999.

What are my alternatives if I choose not to have this treatment?

Ask about alternatives for your exact stage and tumour results, including other anticancer treatments, trials, surveillance where appropriate and symptom-focused care. Supportive care remains available whether or not you proceed.

Does an abnormal blood test mean pembrolizumab must be stopped permanently?

Not by itself. Your team checks whether the change is caused by immune inflammation, another medicine, cancer or another illness. They decide whether to pause treatment, investigate further or stop it. Do not change the treatment plan yourself.

> **Try explaining the plan back:** Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Lung cancer information This patient suite covers the two lung indications described here. The shared clinical protocol also lists other cancers, which need their own patient information.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pembrolizumab 400 mg every 6 weeks

Active medicines: pembrolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis or routine infusion premedication is usually required. Use an individual/local symptom-relief prescription if needed.
- Carry the immunotherapy alert card and report possible immune symptoms promptly during and after treatment.
- Blood count, renal/liver profile and glucose before each cycle; thyroid every 3–6 weeks, cortisol if clinically indicated.
- Effective contraception during treatment and at least 4 months after the last dose.
- If treatment is paused because of an immune side effect, your team checks recovery and any steroid or hormone plan before deciding whether it can restart. Do not change steroids or restart treatment yourself.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / nervous system

Pembrolizumab

Severe headache, confusion, weakness, dizziness or marked thirst need prompt advice.

3 Heart

Pembrolizumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

5 Kidneys

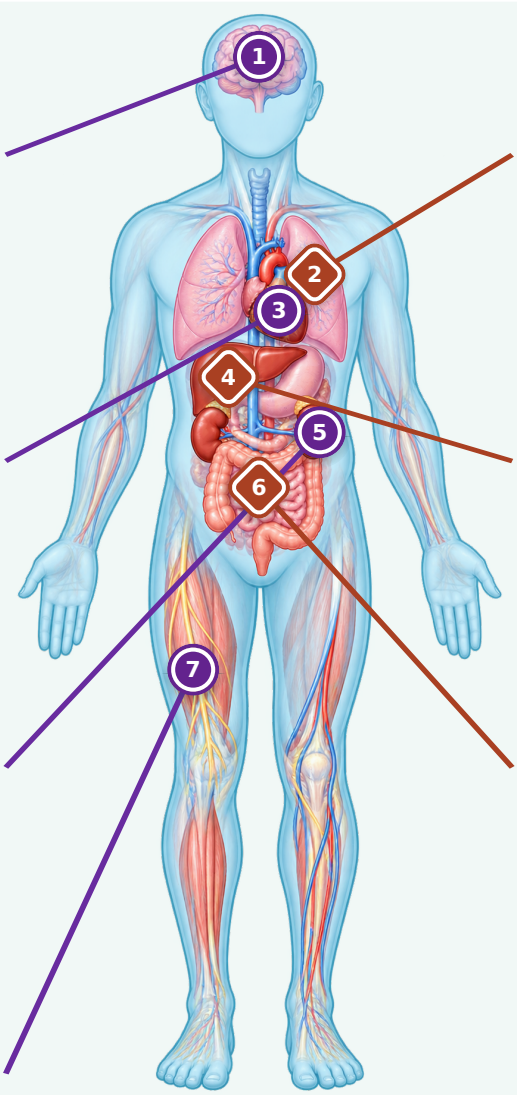
Pembrolizumab

Report much less urine, new swelling or a change in kidney tests.

7 Skin / muscles

Pembrolizumab

Spreading rash, blisters, severe weakness or muscle pain needs prompt review.



2 Lungs

Pembrolizumab

New cough or breathlessness can signal inflammation or another urgent cause.

4 Liver

Pembrolizumab

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

6 Bowel

Pembrolizumab

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team immediately for new or worsening breathlessness, chest pain, persistent diarrhoea, severe tummy pain, yellow skin/eyes, much less urine, severe headache, marked weakness, confusion, unusual thirst or passing much more urine, or a spreading/blistering rash. For severe breathing difficulty, collapse, seizure or other immediately dangerous symptoms, seek emergency care. Do not wait for the next infusion.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00558 v13 · official regimen**
   https://healthservice.hse.ie/documents/6389/Pembrolizumab_400mg_monotherapy_558_.pdf
2. **EMA Keytruda intravenous (IV) product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/keytruda-epar-product-information_en.pdf
3. **KEYNOTE-024 five-year randomized follow-up · Reck et al. 2021**
   https://pubmed.ncbi.nlm.nih.gov/33872070/
4. **PEARLS/KEYNOTE-091 randomized adjuvant study · O'Brien et al. 2022**
   https://pubmed.ncbi.nlm.nih.gov/36108662/
5. **Lala et al. 2020 · 400 mg six-week exposure-bridging study (with corrigendum)**
   https://pubmed.ncbi.nlm.nih.gov/32305010/
6. **HSE · chemotherapy safety and when to contact your team**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
7. **EORTC primary trial-sponsor report · PEARLS/KEYNOTE-091 final analysis, ESMO 2026**
   https://www.eortc.org/blog/2026/10/27/eortc-at-esmo-2026/